



SURGICAL PATHOLOGY REPORT

Patient Name: [REDACTED]
Med. Rec. #: [REDACTED] Visit #: [REDACTED]
DOB: [REDACTED] Sex: Male
Soc. Sec. #: [REDACTED] Location: [REDACTED]
Physician(s): [REDACTED]

Accession #: [REDACTED]
Service Date: [REDACTED]
Received: [REDACTED]
Client: [REDACTED]

FINAL PATHOLOGIC DIAGNOSIS

*ICD O-3
Carcinoma, urothelial papillary
8132/3
Site Bladder, lateral wall
0672
10/31/14*

A. Ureter, left, biopsy: No carcinoma.

B. Ureter, right, biopsy: No carcinoma.

C. Bladder and prostate, cystoprostatectomy:

- Urinary bladder:

1. High-grade urothelial carcinoma with micropapillary features, invading through muscularis propria into perivesicular adipose tissue.
2. Mucosal ulceration with granulomatous inflammation and calcification.
3. No carcinoma in one perivesicular lymph node (0/1).

- Prostate: No carcinoma.

- Seminal Vesicles: No carcinoma.

D. Lymph nodes, right pelvic, excision: Metastatic carcinoma in one of nine lymph nodes (1/9).

E. Urethra, final margin, biopsy: No carcinoma.

F. Lymph nodes, left pelvic, excision: Metastatic carcinoma in three of seven lymph nodes (3/7).

G. Ureter, left, biopsy: No carcinoma.

H. Ureter, right, biopsy: No carcinoma.

COMMENT:

Bladder Tumor Synoptic Comment

- **Tumor type:** Urothelial carcinoma with micropapillary features and focal squamous and glandular differentiation.
- **Tumor grade:** High grade.
- **Tumor size:** 4.2 x 1.7 x 1.6 cm.
- **Extent of tumor in bladder:** Invasion through muscularis propria into perivesical tissue.
- **Lymphatic/vascular invasion:** Present in lymphatics.
- **Epithelial abnormalities in bladder:** Carcinoma in situ.
- **Extension of tumor into adjacent organs:** None.
- **Margins:**
 - **Urethral margin:** Negative (invasive tumor is greater than 1 cm from margin; dysplasia/CIS is greater than 1 cm from margin). Final urethral margin specimen (part E) is negative for carcinoma.
 - **Right ureter margin:** Negative (invasive tumor is greater than 3.1 cm from margin; dysplasia/CIS is greater than 3.1 cm from margin). Final right ureter margin specimen (part H) is negative for carcinoma.
 - **Left ureter margin:** Negative (invasive tumor is greater than 0.8 cm from margin; dysplasia/CIS is greater than 0.8 cm from margin). Final left ureter margin specimen (part G) is negative for carcinoma.
 - **Perivesical margin:** Negative (invasive tumor is 0.2 cm from margin (left lateral, slide C4; dysplasia/CIS is greater than 1 cm from margin).
- **Lymph node status:** Positive
 - total number of positive nodes: 4.
 - total number of nodes examined: 17.
 - Size of largest metastasis in node: 0.8 cm.
 - Extranodal extension: None.
- **Other pathologic findings in bladder:** Granulomatous inflammation, ulcer, calcifications.
- **AJCC/UICC stage:** pT3aN2.

Level sections were obtained and evaluated to confirm the diagnosis.

Specimen(s) Received

A:Ureter, biopsy (FS)
B:Ureter, biopsy (FS)
C:Bladder and prostate (FS on urethral apex)
D:Lymph node, right pelvic
E:Final urethral margin
F:Lymph node, left pelvic
G:Distal left ureter
H:Distal right ureter

Intraoperative Diagnosis

FS1 (A) Left ureter, biopsy: No carcinoma.

FS2 (B) Right ureter, biopsy: No carcinoma.

FS3 (C) Urethral margin on bladder and prostate, biopsy: No carcinoma.

Clinical History

The patient is a -year-old man who, per the electronic medical record has high-grade, T2 transitional cell carcinoma diagnosed on transurethral resection of bladder tumor on . He is now undergoing a radical cystoprostatectomy and bilateral pelvic lymph node dissection.

Gross Description

The specimen is received in eight parts, each labeled with the patient's name and medical record

number. Parts A-C are received fresh. Parts D-H are all received in formalin.

Part A, labeled "left ureter," consists of a single soft, irregular piece of off-white/pink tissue (0.4 x 0.4 x 0.2 cm). The entire specimen is frozen for frozen section diagnosis 1 and subsequently submitted in cassette A1.

Part B, labeled "right ureter," consists of a single soft, irregular piece of off-white pink tissue (0.4 x 0.4 x 0.2 cm). The entire specimen is frozen for frozen section diagnosis 2 and subsequently submitted in cassette B1.

Part C is labeled "bladder and prostate," and consists of a cystoprostatectomy specimen (664 gm; 11.3 cm from right to left x 6.8 cm from anterior to posterior x 23 cm from superior to inferior).

GROSS PATHOLOGIC FINDINGS: There is a soft, tan-pink, friable, papillary lesion (1.6 cm from right to left x 4.2 cm from superior to inferior x 1.7 cm deep) apparently extending into the perivesicular fat. The lesion is centered above the left trigone area, abutting the left ureteral orifice, and extends superiorly up the left lateral wall to 1.1 cm below the dome of the bladder. The muscular invasion appears to extend from the left lateral to the right lateral wall just past the midline, and in the right lateral wall involving a portion of the muscle that is 3.5 x 1 x 0.8 cm, and not extending into the fat of the right lateral bladder wall. The main lesion is 1 cm from the urethral orifice, 0.8 cm from the left ureteral orifice, 3.1 cm from the right ureteral orifice, and 0.9 cm at the closest approach to the perivesicular fat margin (left, blue); 0.5 cm from the right external perivesicular margin the right lateral wall portion of the lesion. The lesion does not involve the ureteral orifice. The surrounding bladder mucosa is pink, smooth, boggy, and edematous with punctate hemorrhages in the trigone area. The attached prostate is pink-tan and lobular with nodules ranging from 0.5-0.8 cm in greatest dimension, mostly in the anterior portion of the gland.

The ureters are not attached. The attached perivesicular and periprostatic fat is carefully searched for lymph nodes.

ORIENTED BY: Anatomic landmarks.

INTRAOPERATIVE EVALUATION: Frozen section of left ureter, en face (frozen section 1), submitted as A1. Frozen section of right ureter margin, en face (frozen section 2), submitted as B1. Frozen section of bladder and prostate at apex (frozen section 3), submitted as C1.

INKING:

- Anterior right surface: Green.
- Anterior left surface: Blue.
- Posterior surface: Black.

GROSS PHOTOGRAPHS: Yes.

CASSETTES: Representative sections are submitted as follows:

- C1: Bladder and prostate frozen section remnant.
- C2-C3: Lesion, left lateral bladder wall, deepest extent of invasion, including deep margin (blue), bisected (bisected surface is black).
- C4: Lesion, left lateral bladder wall, representative section (1.8 cm from ureteral orifice).
- C5: Lesion, left lateral bladder wall, with approach to left ureteral orifice.
- C6: Lesion, left lateral bladder wall, with approach to bladder dome.
- C7: Lesion, left lateral bladder wall, with approach to normal mucosa laterally.
- C8: Lesion, right lateral bladder wall, closest approach to deep perivesicular margin (black).
- C9: Lesion, with closest approach to deep black margin (0.8 cm).
- C10: Left ureteral margin, en face.
- C11: Right ureteral margin, en face.
- C12-C13: Uninvolved right ureteral orifice, bisected (perpendicular section).
- C14: Lesion with approach to left ureter, and deep margin (perpendicular section).
- C15: Bladder dome, apparently normal.
- C16: Right lateral bladder wall, apparently normal.
- C17: Lesion, posterior bladder wall.
- C18: Lesion, left lateral bladder wall, with closest approach to urethral orifice (perpendicular

- section).
- C19: Lesion, left lateral bladder wall, closest approach to prostate (parallel section).
- C20: Prostate, prostatic urethral margin, en face.
- C21: Prostate, right anterior and posterior apical one-third of gland.
- C22: Prostate, right anterior and posterior middle third of gland.
- C23: Prostate, right anterior and posterior base of gland.
- C24: Prostate, left anterior and posterior apical one-third of gland.
- C25: Prostate, left anterior and posterior middle third of gland.
- C26: Prostate, left anterior and posterior base of gland.
- C27-C28: Candidate lymph nodes, right perivesicular fat.
- C29-C30: Candidate lymph nodes, left perivesicular fat.
- C31: Seminal vesicle, left, representative section.
- C32: Seminal vesicle, right, representative section.

Part D is additionally labeled "right pelvic lymph nodes," and consists of multiple soft to firm, irregular, unoriented, tan-yellow/brown, fibrofatty tissue fragments (8.5 x 8 x 4 cm in aggregate). Multiple candidate lymph nodes (0.6-5.4 cm in greatest dimension) are found. The largest candidate lymph node has mostly pink-white tissue mixed with homogeneous, yellow parenchyma. The second-largest candidate lymph node (3.5 cm in greatest dimension) has pink-yellow/red parenchyma. The remaining candidate lymph nodes are left intact. Representative sections (but all candidate lymph nodes) are submitted in cassettes as follows:

- D1-D4: Largest candidate lymph node.
- D5-D7: Second-largest candidate lymph node.
- D8: Two intact candidate lymph nodes.
- D9: Four intact candidate lymph nodes.

Part E is additionally labeled "final urethral margin," and consists of a single soft to firm, slightly ovoid, unoriented, tan-white/pink tissue fragment (1.5 x 0.8 x 0.5 cm). There is an off-centered lumen (internal luminal diameter is 0.2 cm) that is patent and present. The margin is inked blue. The entire specimen is submitted in cassette E1.

Part F is additionally labeled "left pelvic lymph node," and consists of multiple soft to firm, irregular, unoriented, tan-yellow/brown, fibrofatty tissue fragments (7.5 x 7.5 x 2.8 cm in aggregate). Multiple candidate lymph nodes (0.3 cm to 8 cm in greatest dimension) are found. The largest candidate lymph node has firm, pink, surrounded by yellow, parenchyma. The second-largest candidate lymph node (1.6 cm in greatest dimension) has firm, white-pink parenchyma. The remaining candidate lymph nodes are left intact. Representative sections (but all candidate lymph nodes) are submitted in cassettes as follows:

- F1-F6: Largest candidate lymph node.
- F7: Second-largest candidate lymph node.
- F8: Three intact candidate lymph nodes.
- F9: Four intact candidate lymph nodes.

Part G is additionally labeled "distal left ureter," and consists of a single unoriented, tubular, tan-brown tissue fragment (1.2 cm in length x 0.4 cm in diameter). The lumen is patent (0.2 cm internal luminal diameter). One-half of the tissue is inked blue and the other half is inked black. The entire specimen is submitted in cassette G1.

Part H is additionally labeled "distal right ureter," and consists of a single unoriented, tubular, tan-brown tissue fragment (2.6 cm in length x 1.5 cm in diameter). The lumen is patent (0.2 cm internal luminal diameter). One-half is inked blue with the other half inked black. The entire specimen is submitted in cassettes H1 and H2.

Diagnosis based on gross and microscopic examinations. Final diagnosis made by attending pathologist following review of all pathology slides.

Pathology Resident

Electronically

Criteria	Yes	No
Diagnosis Discrepancy		
Primary Tumor Site Discrepancy		
HIPAA Discrepancy		
Prior Malignancy History		
Dual/Synchronous Primary Noted		
Case is (circle):	QUALIFIED	DISQUALIFIED
Reviewer Initials	JAC	1224/2013